



POLICY

Clinical Engagement

Scope (Staff):	All Staff
Scope (Area):	ICAMHS

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Contents

Aim	1
Risk.....	2
Background.....	2
Definitions	2
Principles	3
Procedure	3
Factors to consider in Engagement	3
Actions for Non-Engagement.....	4
Factors to consider when a child and family do not attend (DNA) or unexpectedly attend appointments.....	4
For children who are under the protection of the Department of Communities, Child Protection and Family Support	6
Discharge as a result of non-engagement or non-attendance.....	6
Documentation.....	7
Implementation	7
Compliance.....	7

Aim

To provide guidance to clinicians of the Child and Adolescent Mental Health Service (CAMHS) on;

- the use of recovery-orientated practice to support engagement;
- the application of the empirical evidence relevant to establishing a strong therapeutic relationship;
- consistent responses to unattended appointments or unexpected attendances; and
- the development and implementation of strategies for high risk children with poor family and systemic supports who do not attend appointments.

Risk

- Not identifying and addressing the factors affecting engagement and non-attendance of children and families.
- Poor clinical outcomes.
- Negative impact of unattended appointments on the booking system, including unavailability of appointments for other children and families.

Background

Engaging individuals living with serious mental illness in ongoing treatment can often be difficult. Non-attendance may indicate poor engagement, which is a significant clinical risk. Given a significant number of CAMHS children and families terminate treatment prematurely, and the quality of engagement is positively correlated to clinical outcomes, it is important to identify and address any factors which might be affecting engagement.

Poor engagement can be due to distress, family chaos, child neglect, or the lack of a strong therapeutic relationship (also known as therapeutic alliance). Disengagement may be related, but not limited, to numerous variables including; issues regarding perceived and/or actual effectiveness of treatment; expectations of the child and family; attitudes such as mistrust; or practical reasons such as accessibility of care. Experiences of services and the level of child and family satisfaction can also influence engagement of children and families and treatment completion.

The therapeutic alliance, which can be enhanced by a recovery orientation, is important in facilitating engagement in care. A trusting therapeutic relationship affects the willingness of people to seek care, reveal sensitive information and engage in treatment. The use of person-centred care, which incorporates shared decision making and addresses an individual's immediate needs, has promising outcomes for engagement.

Definitions

Child: an infant, child, adolescent or young person under the age of 18 years.

Family: any parent, legal guardian, or identified carer of the child. For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer. For a

legal definition, refer to the [Mental Health Act 2014](#), section 281 - Close Family Members.

Did Not Attend (DNA): when a child and family do not attend an appointment (and do not contact the service to cancel the appointment) or continuously do not attend and/or reschedule appointments at short notice.

Structured clinical judgement: judgement about risk based on a combination of; the evidence base for risk factors; individual patient assessment including the child and family's view of their own experience and circumstances; clinical experience; and knowledge of the consumer.

Therapeutic relationship: the relationship between the case manager and/or clinician and the child and family who are all engaged in working together to promote recovery.

Principles

- The [Charter of Mental Health Care Principles](#) informs CAMHS services.
- A structured clinical judgement approach guides assessments and reviews.
- CAMHS clinicians must be familiar with the empirical literature relevant to the therapeutic relationship. The [National Framework for Recovery-Oriented Mental Health Services](#) provides a guide for clinicians.
- CAMHS clinicians must ensure that the services they offer are inclusive, flexible, child-centred and recovery-focussed and consider the child's age and developmental stage or needs.
- Recovery-oriented principles are about putting the child at the centre of practice and service delivery and viewing a child's life situation and development needs holistically, in order to guide care.
- Children and their families are recognised by CAMHS clinicians as being part of a wider community. Mental health services are viewed as one part of a wider service network.
- Clinicians support the provision of coordinated and integrated care across programs sites and services in accordance with the [CAMHS Access Policy](#).

Procedure

Factors to consider in Engagement

- Timeliness of care with appropriate clinical prioritisation of referrals.
- Using all methods of communication, including phone (landline or mobile using phone call or text), email, mail and /or contact through third parties such as the referrer or other stakeholders.
 - Consider the child and family's preference for type of communication where possible.
- Meeting the diverse needs related to individual and family circumstances.

- The time, day and frequency of appointments and how they are booked with the child and family (i.e. opting in for first appointment, forward booking at the end of the appointment or on the phone).
- Wherever possible offer appointments at a venue suitable to the child and their family.
- Matching clinicians to child and family needs in accordance with the [CAMHS Access Policy](#).
- Promoting or maximising the therapeutic relationship between clinician, child and family.
- Carefully supporting clients and families if their usual clinician is not available (e.g. because of illness or absence for another reason).
 - If a clinician is unwell or absent, the Head of Service (or proxy) should review their appointments and new referrals and arrange for rescheduling/reallocation as indicated by acuity and clinical context.
- Assessing suitability of method of service delivery, while also considering risk. Where there is an identified need consider:
 - Meeting in the local community, for example at the school.
 - Home visits.
 - Telehealth - For details of important things to consider when using telehealth, including record keeping, privacy and risk considerations, please see the [Use of Telehealth for Mental Health Assessment and Examinations](#).
 - Referral to a community service closer to home as appropriate, possibly as a shared care arrangement.
 - Other strategies in line with recovery-oriented care.

Actions for Non-Engagement

- Clinicians must present the case at the Multidisciplinary Team (MDT) review meeting.
- The MDT must consider possible causative factors and barriers to engagement and explore alternative approaches to promote better engagement.
 - Outcomes must be documented at the MDT review meeting and filed in the medical record and PSOLIS.
- Consultation and collaboration with referrers and stakeholders must take place. If this is not possible, reasons must be documented in the child's medical record and PSOLIS.

Factors to consider when a child and family do not attend (DNA) or unexpectedly attend appointments.

- Review the patient information to make a structured clinical judgement about risk; capacity; child, family and systemic supports; and the strength of the therapeutic relationship.
 - See the [Principles and Best Practice for the Care of People Who May Be Suicidal document](#).
 - Review the clinical history and risk through referral information, PSOLIS entries, recent assessment and reviews as per [CAMHS Risk Assessment and Management Policy](#) and determine the level of risk. This should include physical healthcare risks.
 - Assess previous engagement with other CAMHS services.
 - For Community CAMHS: The child's medical record from previous CAMHS episodes must be accessed. A new medical record must not be created if the child has previously been seen by another Community CAMHS team.
 - Decide whether there is a functioning and a well-resourced support system in place for the child and/or family (such as extended family and friends, school, GP, private practitioners).
- If the CAMHS clinician and/or Case Manager has **significant or immediate** concerns:
 - Attempt to make contact via phone.
 - If contact is made via phone, clinical staff should make all efforts to speak with the client directly (as developmentally appropriate), as well as their carer/s. This will improve the opportunity to accurately assess the client's current state, including risk.
 - Immediately inform the most senior clinician to collaboratively develop an action plan which may include assertive outreach; contacting the Department of Communities, Child Protection and Family Support and/or WA Police.
 - Implement the action plan and document it in the child's medical record and PSOLIS.
 - Clearly document all the activities and outcomes which have been undertaken in accordance with the action plan in the child's medical record and PSOLIS.
- If the CAMHS clinician and/or Case Manager has **no significant** concerns or **minimal** concerns:
 - Attempt contact by telephone or other means as soon as possible but no later than 5 working days to find out if there are any concerns / issues and to reschedule the appointment.
 - If face-to-face or telephone contact is not established, other forms of contact must be tried for example text messages. A letter must then be sent

inviting the child / family to call and make another appointment within 14 days. This letter must be copied to the referrer and other stakeholders as appropriate and where there is consent.

- Where contact has not been made within 14 days, the case must be discussed at the MDT review meeting for decision regarding next steps (for example deactivation, more attempts to contact and engagement required). When a further attempt to contact and engage is made, the same process applies.
- When there is no contact or engagement following these attempts, the case must be taken to the MDT review meeting and discharge endorsed. See the [CAMHS Clinical Handover Guideline](#).
- All actions must be documented in the child's medical record and on PSOLIS.
- The process for **unexpected attendance** is as follows:
 - The Administration staff member/receptionist must contact the Case Manager to let them know that the child and family are at the clinic. If the Case Manager is not available, the Choice Coordinator or another senior clinician, and then the Service Manager or Head of Service must be contacted.
 - The available clinician (or manager) must make a structured clinical judgement of risk to decide what actions need to be taken and whether the child and family should be seen and assessed.
 - The decision-making process and outcome must be documented in the child's medical record and in PSOLIS as a service event by the clinician/manager who made the decision. This must include the clinical judgement reasons.

For children who are under the protection of the Department of Communities, Child Protection and Family Support

- Any initial queries around engagement and attendance should be discussed within the MDT review setting.
- Child Protection Consultation Liaison (CPCL) Officers are the CAMHS portfolio holders and can be accessed for any queries relating to this group of children.
- Queries around engagement and attendance should be discussed with the CPCL Officers.
- Local metropolitan CAMHS and CPFS consultation-liaison meetings can be used to discuss ongoing issues regarding engagement as a complex matter.

Discharge as a result of non-engagement or non-attendance

- The decision to discharge or close a case due to non-attendance must only be taken once there has been assertive efforts made to engage with the child and family, and when all known risks have been considered by the MDT.

- The referrer and other relevant agencies (where the child / family have previously provided consent) must be informed of the rationale for discharge by letter within seven (7) days of this decision. This must include information on how to re-refer the child if required.
- The child and family must be advised by letter of the decision to discharge / close the case within seven (7) days of MDT decision with advice and information regarding other available services and how to be re-referred to CAMHS if required.

Documentation

- Actions taken after non-attendance must be documented in the medical record and on PSOLIS.
- Actions taken after unexpected attendance must be documented in the medical record and on PSOLIS.
- Decisions made at the MDT review meeting regarding attendance or engagement must be recorded in the child's medical record and the recommended strategies addressing poor engagement or attendance must be recorded in the PSOLIS management plan.
- Inform the receiving service of the closure / discharge in writing as well as the referrer, GP and the family, where appropriate. Include a copy of the CAMHS Care Transfer Summary with consent. Care must be taken with exchange of information regarding related parent information.
- Record actions in CAMHS Care Transfer Summary, Discharge NOCC, PSOLIS De-activation for active clients and on the Referral Management & Choice Form for referred clients.

Implementation

Support available for staff in the implementation of this procedure is via the following;

- Head of Service and Service Manager.
- Clinical skills training.
- Clinical supervision.
- MDT review meetings.
- Access to specialist clinicians, such as CAMHS Aboriginal Mental Health Workers and CAMHS Specialised teams.

Compliance

Compliance with this policy will be audited using the clinical documentation audit.

Related CAHS internal policies, procedures and guidelines
CAMHS Access Policy
CAMHS Risk Assessment and Management Policy
CAMHS Clinical Handover Guideline
State-wide Standardised Clinical Documentation (SSCD) CAMHS Forms
References and related external legislation, policies, and guidelines
Mental Health Act 2014
Charter of Mental Health Care Principles
National Framework for Recovery-Oriented Mental Health Services
Use of Telehealth for Mental Health Assessment and Examinations
Principles and Best Practice for the Care of People Who May Be Suicidal document
Specialist Outpatient Services Access Policy – Metropolitan Health Services Policy

This document can be made available in alternative formats on request.

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Standards Applicable:	NSQHS Standards:  NSMHS: 1, 2, 3, 4, 6, 9, 10 Child Safe Standards: 1, 2, 3, 4, 5		
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 Healthy kids, healthy communities CompassionExcellenceCollaborationAccountabilityEquityRespect Neonatology Community Health Mental Health Perth Children's Hospital			

*Note: CAMHS Director Clinical Services approved a 12-month extension of the review date to 25/05/2026.